

Treat the patient needing palliative care

- If pain, aim to have patient pain free at rest, able to sleep and manage daily tasks. Start pain medication based on type and severity of pain:

Non-cancer pain

- If **mild** (1-3) pain, start at step 1.
- If **moderate** (4-7) or **severe** (8-10) pain start at step 2.
- If **nerve pain** or **central pain**, also give amitriptyline at any step.

Cancer pain

- If **mild** (1-3) pain, start at step 1.
- If **moderate** (4-7) pain start at step 2. If **severe** (8-10) pain start at step 3.
- Also consider adding amitriptyline at any step.

If unsure, start at lower step and increase pain medication if needed.

- If pain controlled, continue same dose. Once controlled for 1 month, consider reducing dose/stepping down. If pain worsens, then increase dose/step up again.
- If pain persists > 2 days or worsens, increase dose to maximum. If still no better after 2 days, move to next step.
- If pain not responding well, assess and provide additional social, spiritual and emotional support ➔ 61.
- If non-cancer pain uncontrolled on step 2, refer. If cancer pain uncontrolled on step 3, discuss.

Step	Pain medication	Start dose	Maximum dose	Note
Step 1 Start one or both of:	Paracetamol Ibuprofen	1g 4-6 hourly 400mg 8 hourly	4g daily 1.2g daily	If starting, give paracetamol 1g in clinic and reassess pain after 4 hours. If no better, add ibuprofen . • If starting step 1 with ibuprofen, reassess pain after 4 hours. If no better, add paracetamol. • Give with/after food. Avoid if peptic ulcer, asthma, hypertension, heart failure or kidney disease. • If also taking aspirin, advise to take at least 30 minutes apart.
Step 2 Add to step 1:	Tramadol	50mg 4-6 hourly	400mg daily	• If constipation or nausea/vomiting, manage as below. • Use with caution if patient on amitriptyline as may cause over-sedation.
Step 3 (only if cancer pain) Stop tramadol, continue paracetamol/ibuprofen and add:	Morphine hydrochloride (short-acting, solution) or Morphine sulphate (long-acting, tablet)	5-10mg 4 hourly If ≥ 65 years: start 2.5-5mg 10-20mg 8-12 hourly	• No maximum-titrate against pain. • If sedated/confused or respiratory rate < 12, skip 1 dose, then halve usual doses.	• Start with morphine hydrochloride. Once dose stable, consider changing to long-acting morphine sulphate. • Also give lactulose 10-20mL daily to prevent constipation. Avoid if diarrhoea. • If constipation, nausea/vomiting or itchiness, manage as below. • If on morphine hydrochloride and breakthrough pain (pain that occurs before next scheduled dose): - Give one extra dose morphine, then continue regular dose at scheduled times for the rest of that day. - Increase morphine doses the next day. Calculate new dose: add up total amount of extra morphine given in last 24 hours. Divide this amount by 6 and add this to each regular 4 hourly dose ¹ .
Add adjuvant therapy to any step	Amitriptyline	• 25mg at night • If ≥ 65 years: 10mg	75mg at night	• Use at night. Advise it may cause dizziness, drowsiness and to avoid driving and using heavy machinery. • Avoid if on bedaquiline, refer/discuss if pain uncontrolled on above medication.

Treat side effects from pain medication or other symptoms

Constipation	Diarrhoea	Nausea/vomiting	Abdominal cramps	Generalised itchiness	Acute anxiety	Cough or difficulty breathing
• Check for impaction (solid bulk of stool in rectum). If impacted, gently remove stool from rectum using lubrication. If unsuccessful, refer. • Give sennosides A and B 13.5mg at night and/or lactulose 10-20mL orally daily. If needed, increase sennosides A and B to 27mg at night and/or increase lactulose to 12 hourly. If no response, refer.	Give loperamide 4mg initially, then 2mg after each loose stool up to 6 hourly, up to 12mg daily. Avoid if overflow diarrhoea or side effect of antibiotics.	• Give metoclopramide 10mg 8 hourly as needed. • Allow patient to choose what to eat. Encourage frequent small meals/sips of fluids like water, tea or ginger drinks.	Give hyoscine butylbromide 10mg 6 hourly as needed for up to 3 days.	Give chlorphenamine 4mg 6-8 hourly as needed.	Give diazepam 2.5-5mg 12 hourly as needed for up to 10 days.	• If thick sputum, give steam inhalations. If more than 30mL/day, try deep breathing with postural drainage. Refer to physio if available. • If excess thin sputum or persistent dry cough, discuss with palliative care specialist. • If low oxygen saturation, refer for home-based oxygen.

Review 2 days after starting or changing medication. If pain/symptoms persist despite treatment or side effects intolerable, discuss/refer.

¹Example: patient on morphine 10mg 4 hourly has 3 episodes of breakthrough pain: 10mg x 3 = 30mg (total extra morphine); 30mg ÷ 6 = 5mg. Add 5mg to each 10mg regular dose. Increase morphine to 15mg 4 hourly.